INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Eribulin · Days 1 and 8 of 21

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
21 days
See the active treatment phase

Treatment stages
1
Explained in the treatment reference

Regimen source
NCCP 00228
Version 8

> Eribulin on Days 1 and 8 of each 21-day cycle. Day 15 is a planned treatment-free week. Treatment continues while beneficial and tolerated.

Treatment stages

01 **Eribulin · Days 1 and 8 of 21**
21-day cycle

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

Eribulin · Days 1 and 8 of 21
National Cancer Control Programme (NCCP) 00228

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. New chest pain, fainting, severe breathlessness, fever or sudden illness needs urgent assessment. Report worsening numbness, severe constipation, mouth ulcers or inability to drink promptly.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

How this medicine works

The aim is to control the cancer and its symptoms. The team reviews benefit, side effects and your priorities before continuing.

CHEMOTHERAPY

Eribulin

Halaven · eribulin base



Stops microtubules growing

Eribulin blocks growth of the tiny structures cells need to divide. It can slow cancer growth, but can also affect blood cells and nerves.

Cell-division structures → Growth blocked → Cell division disrupted

What to watch for

Report numbness, weakness or difficulty with walking or hand tasks before the next dose. Fever or sudden illness needs immediate advice; palpitations or fainting need urgent assessment.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

The exact treatment being discussed

Indication

Advanced breast cancer after at least one chemotherapy regimen for advanced disease, with prior anthracycline and taxane unless unsuitable; or adult unresectable liposarcoma after anthracycline unless unsuitable.

Dose and formulation

1.23 mg/m² eribulin base. EU dose labels use eribulin base. 1.23 mg/m² base corresponds to 1.4 mg/m² mesylate in some trial reports; these are not two alternative doses.

Evidence limits

In heavily pretreated advanced breast cancer, eribulin helped people live longer on average than the comparison treatments chosen by their doctors. This does not predict an individual result or establish the same benefit in every treatment line or another cancer. EU dose is 1.23 mg/m² eribulin base; 1.4 mg/m² mesylate in trial reports is the equivalent amount, not an alternative numeric prescription. Day 8 may be delayed or omitted after review.

Eribulin versus dacarbazine: advanced liposarcoma evidence

452 adults with advanced liposarcoma or leiomyosarcoma after at least two systemic regimens including anthracycline, using Days 1 and 8 every 21 days. Separate sarcoma trial and its liposarcoma subgroup support the liposarcoma indication; breast-trial benefit figures do not apply. The licensed liposarcoma indication must not be expanded to all sarcomas because the parent trial also enrolled leiomyosarcoma. Use current NCCP eligibility and EMA indication wording. Do not apply breast imaging or CA 15-3/carcinoembryonic antigen (CEA) pathways to liposarcoma.

Why this treatment may be chosen

The treating team considers the cancer, previous treatment, health and your priorities. Trial averages do not predict an individual outcome.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

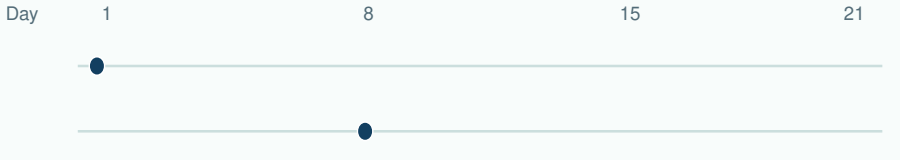
Your treatment stages, checks and practical preparations, from before treatment through follow-up.

Eribulin · Days 1 and 8 of 21
NCCP 00228

Treatment, reassessment and the next decision

Eribulin · Days 1 and 8 of 21

21-day view



This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

21-DAY CYCLE

Eribulin · Days 1 and 8 of 21

Repeat according to benefit and tolerance

Eribulin on Days 1 and 8 of each 21-day cycle. Day 15 is a planned treatment-free week. Treatment continues while beneficial and tolerated.

This card describes one active chemotherapy schedule. There is no automatic maintenance phase; continuing, changing or stopping treatment follows clinical review.

BASELINE → TREATMENT → FOLLOW-UP

When checks happen

Milestones show order, not equal time intervals or booked appointments. The treating team requests tests, reviews results and confirms treatment. full blood count (FBC) before each dose, including Day 8, as required by the Halaven SmPC; renal/liver profile at each cycle start. Review neuropathy and other toxicity before treatment. EU dose is 1.23 mg/m² eribulin base; 1.4 mg/m² mesylate in trial reports is the equivalent amount, not an alternative numeric prescription. Day 8 may be delayed or omitted after review.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00228 v8 · official regimen · EMA Halaven product information

THE WHOLE TREATMENT JOURNEY

Eribulin · Days 1 and 8 of 21: treatment and checks

Previously treated advanced breast cancer or unresectable liposarcoma

Eribulin on Days 1 and 8 of each 21-day cycle. Day 15 is a planned treatment-free week. Treatment continues while beneficial and tolerated.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Early treatment	Ongoing treatment	Dose change / pause	Next decision
Treatment plan NCCP 00228	Confirm indication, goal and current prescription	Days 1 and 8 each 21 days while helpful and tolerated	Continue according to response, tolerance and prescribed course	Team review before restart or dose change	Agree completion or next treatment
Blood / kidney / liver NCCP 00228 · SMPC	Baseline FBC, kidney and liver tests	FBC before every dose; kidney/liver at cycle start	FBC before every dose; kidney/liver at cycle start	Recheck recovery and dose before restarting	Follow unresolved abnormalities
Symptoms / medicines NCCP 00228 · SMPC	Review symptoms, medicines and previous treatment	Review effects before each dose	Continue symptom and medicine reviews	Report problems; do not change or restart doses yourself	Agree recovery and next treatment plan
Heart / electrocardiogram (ECG) / echo NCCP 00228 · SMPC	Risk-directed ECG and electrolytes; no automatic three-month echo schedule for eribulin alone.	Review symptoms and indicated tests	Continue the risk- and treatment-specific plan	Urgent assessment for chest pain, fainting or breathlessness	Agree follow-up from exposure and results
Response / course NCCP 00228 · SMPC	Confirm cancer-specific treatment aim	Days 1 and 8 each 21 days while helpful and tolerated	Review benefit, symptoms and the agreed scan plan	Reassess if treatment is held or symptoms change	Agree continuation, change or completion
Bone health / other care NCCP 00228 · SMPC	Review previous/ ongoing treatments and fracture risk	No automatic chemotherapy-cycle dual-energy X-ray absorptiometry (bone density) (DXA)	Keep any separate endocrine/ fracture-risk plan	Review if clinical risks change	Continue separately indicated follow-up

What each check is for

1



Treatment and safety review

Before each planned treatment or four-week review for olaparib

FBC before each dose, including Day 8, as required by the Halaven SmPC; renal/liver profile at each cycle start. Review neuropathy and other toxicity before treatment.

Ask / **act:** The treating team confirms results and the plan; this map does not authorise a dose.

2 **Benefit and continuation**

At agreed clinical review points

In heavily pretreated advanced breast cancer, eribulin helped people live longer on average than the comparison treatments chosen by their doctors. This does not predict an individual result or establish the same benefit in every treatment line or another cancer.

Ask / **act:** Discuss benefit, toxicity and priorities before continuing or changing treatment.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Your infusion and access plan



STEP 1

Before the first visit

Confirm the indication, baseline bloods, required cardiac checks, medicines review and consent before the first infusion.



STEP 2

Plan venous access

A cannula, peripherally inserted central catheter (PICC) or implanted port may be used. If a port is chosen, a trained team inserts it in a separate procedure with local anaesthetic and sometimes sedation; the team explains fasting, blood-thinner and wound instructions.

STEP 3

Infusion day

Eribulin is given by intravenous (IV) infusion over 5 minutes; blood review, preparation and access make the full visit longer. Staff observe for reactions. Report burning, pain, swelling or leakage at the infusion site immediately.

STEP 4

Home and next review

This regimen does not routinely need a take-home infusion pump. Staff disconnect the infusion; a port is de-accessed when appropriate. Keep line-care and next-visit instructions.

Port insertion and removal

The line team explains placement, dressing care and when normal activities can resume. A port is accessed with a special needle during treatment. Removal of the implanted device is a separate planned procedure when no longer needed; do not remove or adjust it yourself.

PICC care and urgent line symptoms

A PICC usually needs regular dressing and flushing appointments even between treatments. Fever, redness, swelling, pain, leakage or damage requires prompt advice. Your hospital supplies the exact care instructions.

Visit length

Eribulin is given by IV infusion over 5 minutes; blood review, preparation and access make the full visit longer. A port is optional access, not a requirement for a take-home pump. This regimen has no routine ambulatory pump.

Premedication

No routine hypersensitivity premedication is specified. Low-risk antiemetic support is reviewed separately, including QT interactions.

Checks throughout treatment

Ask who arranges each test and who reviews it.

Bloods and symptoms

FBC, renal and liver profile before starting and at the start of each cycle. The EMA product information specifies FBC before every dose, including Day 8; additional renal / liver tests follow clinical need.

ECG and heart function

ECG before starting and during treatment when there is cardiac disease, a slow / abnormal rhythm, QT-prolonging medicines or electrolyte disturbance. Correct low potassium, calcium or magnesium before treatment; repeat monitoring follows risk and findings. No fixed routine echo interval is specified for eribulin alone. Cardiac symptoms or risk prompt individual assessment. If trastuzumab is separately prescribed, use its own cardiac-monitoring plan.

Cancer response

For metastatic breast cancer, ESMO recommends response evaluation generally every 2–4 months, adjusted to disease, treatment and symptoms. Agree the scan type, due window and result-review appointment; symptoms can prompt earlier assessment. For the other cancer indications on this card, use the cancer-specific monitoring plan; the breast interval is not transferred automatically.

Markers

Tumour markers, when useful for the cancer being treated, supplement clinical assessment and imaging. A marker result alone should not determine treatment failure or a treatment change.

How other care fits around chemotherapy

There is no automatic surgery, radiotherapy or maintenance phase in this card.

Surgery and radiation

These are advanced-disease regimens. Surgery or radiotherapy is used only for a defined local or symptom-control purpose. The oncology and radiation teams decide whether chemotherapy continues, pauses or precedes radiation; no routine overlap is implied.

Bone health

dual-energy X-ray absorptiometry (bone density) (DEXA) is not a routine test for this chemotherapy alone. Assess fracture risk and bone density when indicated by endocrine treatment, ovarian suppression or other clinical risks; repeat according to the relevant plan.

Changing treatment

Discuss benefit, adverse effects, other treatment options and your preferences at each review. New symptoms can bring review forward.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Eribulin · Days 1 and 8 of 21

Day 1 → Day 21: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
Treatment · Day 1	 **Day 1** intravenous (IV)
Treatment · Day 8	 **Day 8** IV
Planned week off	 **Days 15–21** No routine Day 15 dose
Blood counts / infection	 **Days 1–21** Risk varies; fever needs prompt advice
Energy / appetite	 **Days 1–21** May vary and build across cycles
Numbness / tingling	 **Days 1–21** Can build across doses
Rhythm / fainting symptoms	 **Days 1–21** Report promptly whenever they occur

No dates to enter: This chart explains the pattern. The team arranges appointments and tests.

Review milestone: Review symptoms and benefit throughout each cycle; arrange cancer-specific response imaging under the agreed plan.

A delay changes the treatment plan: Do not move, double or make up doses yourself. A delayed infusion does not automatically move an already-due scan or heart test.

Why these bands stay broad: Treatment markers show the planned days. Symptom bands show broad possibilities, not measured risk curves or guaranteed symptom-free days. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00228 v8 · official regimen ↗

There are no guaranteed "safe days". Fever, shivers or suddenly feeling unwell needs prompt advice on any day, including planned treatment-free weeks.

Between-treatment notes

Day 1–21: Your reference cycle

Eribulin on Days 1 and 8 of each 21-day cycle. Day 15 is a planned treatment-free week. Treatment continues while beneficial and tolerated. The team books actual appointments. Planned breaks are not guaranteed symptom-free days.

Support around the medicines

1 **Supportive medicines**

2 **Everyday care**

Keep gently active as tolerated, pace rest and activity, and report nerve symptoms before the next dose.

3 **Tests and appointments**

Ask who books each test, when it is due and who reviews the result. Tell the team if an expected appointment has not arrived.

4 **Correct medicine and schedule**

5 **Review the full medicine plan**

Low emetic risk; choose preventive/rescue medicines after considering QT prolongation and interactions. No routine hypersensitivity premedication is usually required. Review potassium/magnesium and correct abnormalities; monitor ECG when cardiac/QT risks or symptoms warrant it. Severe neutropenia may be managed with granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) under the treating team. Avoid driving or machinery when tired or dizzy.

Practical care between visits

Report problems early, including during planned breaks.

Medicine support

Benefit, course and continuing checks

Heart checks and the full course

Risk-directed ECG and electrolytes; no automatic three-month echo schedule for eribulin alone. EU dose is 1.23 mg/m² eribulin base; 1.4 mg/m² mesylate in trial reports is the equivalent amount, not an alternative numeric prescription. Day 8 may be delayed or omitted after review.

Questions worth asking

When do I need blood tests, an ECG or echo?

FBC, renal and liver profile before starting and at the start of each cycle. The EMA product information specifies FBC before every dose, including Day 8; additional renal / liver tests follow clinical need. ECG before starting and during treatment when there is cardiac disease, a slow / abnormal rhythm, QT-prolonging medicines or electrolyte disturbance. Correct low potassium, calcium or magnesium before treatment; repeat monitoring follows risk and findings. No fixed routine echo interval is specified for eribulin alone. Cardiac symptoms or risk prompt individual assessment. If trastuzumab is separately prescribed, use its own cardiac-monitoring plan.

Does a port mean a pump at home?

No. A port is a route into a vein. Staff disconnect the infusion and remove the port needle after the visit when appropriate; removal of the implanted device is a separate planned procedure.

Try explaining the plan back: Which days are treatment days, what checks are due before the next dose, and who is arranging and reviewing the next scan?

Eribulin

- Low blood counts, infection and sepsis
- Numbness, tingling or weakness, sometimes lasting
- Fatigue, hair loss, nausea and constipation
- Changes to heart rhythm / QT in susceptible people

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Eribulin · Days 1 and 8 of 21 · reviewed supportive care

Active medicines: Eribulin

Emetogenic risk: Low

Medicine	Dose / route	When
Ondansetron	8 mg orally once	Each treatment day only Review QT, constipation and hepatic dosing; reconcile any steroid already prescribed
Metoclopramide	10 mg orally up to three times daily as needed	Subsequent days: duration to be confirmed Assess contraindications, renal function and interactions

Additional phase-specific support

- No routine hypersensitivity premedication is specified. Low-risk antiemetic support is reviewed separately, including QT interactions.
- EU dose labels use eribulin base. 1.23 mg/m² base corresponds to 1.4 mg/m² mesylate in some trial reports; these are not two alternative doses.
- Review interactions, QT risk and electrolytes before selecting an antiemetic; individualise rather than automatically issuing this draft.
- Low emetic risk; choose preventive/rescue medicines after considering QT prolongation and interactions. No routine hypersensitivity premedication is usually required.
- Review potassium/magnesium and correct abnormalities; monitor electrocardiogram (ECG) when cardiac/QT risks or symptoms warrant it.
- Severe neutropenia may be managed with granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) under the treating team. Avoid driving or machinery when tired or dizzy.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Eribulin · Days 1 and 8 of 21

Eribulin on Days 1 and 8 of each 21-day cycle. Day 15 is a planned treatment-free week. Treatment continues while beneficial and tolerated.

Source reconciled; independent clinical, pharmacy and patient/PPI review pending. Selected effects; not a complete medicine leaflet.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

1 Hands / feet

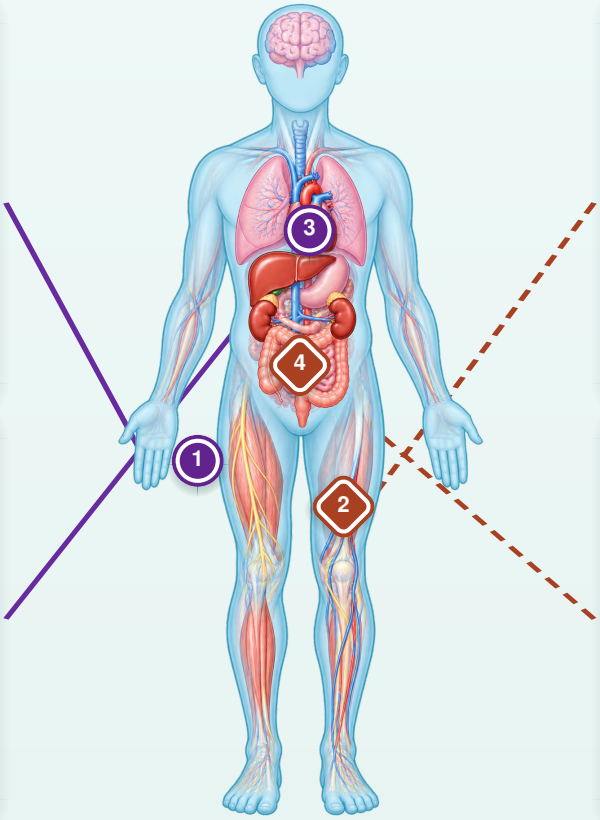
Eribulin

Report numbness, tingling, weakness or loss of function.

2 Blood / infection

Eribulin

Fever, shivers or suddenly unwell: call immediately.



3 Heart rhythm

Eribulin

Fainting or new palpitations need urgent assessment.

4 Bowel

Eribulin

Report persistent constipation, abdominal pain or vomiting.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Selected medicine-specific effects. The full discussion also includes fatigue, hair changes, fertility and other individual risks.

> Fever, shivers or suddenly unwell: contact oncology immediately. Severe chest pain, breathing difficulty, collapse or seizure: 112 / 999.

CHAPTER 04

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **National Cancer Control Programme (NCCP) 00228 v8 · official regimen**
   https://healthservice.hse.ie/documents/6564/228_eriBULin.pdf
2. **EMA Halaven product information**
   https://www.ema.europa.eu/en/documents/product-information/halaven-epar-product-information_en.pdf
3. **Cortes et al. EMBRACE; Lancet 2011; PMID 21376385**
   https://pubmed.ncbi.nlm.nih.gov/21376385/
4. **eviQ · implanted port patient information**
   https://www.eviq.org.au/p/507
5. **eviQ · peripherally inserted central catheter (PICC) patient information**
   https://www.eviq.org.au/p/543
6. **ESC 2022 cardio-oncology guideline · cardiac surveillance and risk**
   https://academic.oup.com/eurheartj/article/43/41/4229/6673995
7. **HSE · chemotherapy side effects and urgent advice**
   https://www2.hse.ie/conditions/chemotherapy/side-effects/
8. **NICE NG 101 · bone health and follow-up**
   https://www.nice.org.uk/guidance/ng101/chapter/Recommendations
9. **NCCP NCIS antiemetics v 8 (2025), Table 1**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf
10. **ASCO · biomarkers in metastatic breast cancer**
    https://ascopubs.org/doi/10.1200/JCO.2015.61.1459
11. **ESMO metastatic breast cancer guideline · 2026**
    https://www.annalsofoncology.org/article/S0923-7534(26)00902-6/fulltext
12. **Schoffski et al. Eribulin versus dacarbazine in advanced liposarcoma or leiomyosarcoma**
    https://pubmed.ncbi.nlm.nih.gov/26874885/
13. **Demetri et al. Liposarcoma subgroup analysis of eribulin versus dacarbazine**
    https://pubmed.ncbi.nlm.nih.gov/28854066/
14. **ESMO Breast Cancer Pocket Guideline 2023**
    https://data.esmo.org/guidelines/pdf/publicassets/ESMO_2023_BreastCancer.pdf
15. **Cardoso et al. ABC 5 consensus: tumour-marker limitations**
    https://pmc.ncbi.nlm.nih.gov/articles/PMC7510449/
16. **NCCP antiemetic guidance; reconcile with regimen and individual prescription**
    https://healthservice.hse.ie/documents/7152/Classification_Document_for_Systemic_Anti-_Cancer_Therapy_systemic anticancer therapy (SACT)_Induced_Nausea__pWgDCrm.pdf